Fat

Fats are given adequately. Increased fat intake at early stages of fever may cause nausea and will be difficult to digest. Medium chain triglycerides (MCT) are preferred and are well tolerated by patients.

Carbohydrate

High carbohydrate diet is prescribed for patients to replenish the glycogen stores and for protein utilization. Well-cooked and easily digestible carbohydrates should be given liberally. These simple carbohydrates will be well assimilated. Examples: glucose, honey, custard, simple starches etc.

Vitamins

B complex vitamin requirements increase with increased energy needs. Antibiotic therapy given for infections alters microbial flora in the intestine. B complex vitamin deficiency may occur. Pyridoxine, Vitamin B_{12} and folic acid should be supplemented for the patients. Vitamin A and C are also essential for wound healing and to maintain the integrity of gut mucosa. Vitamin supplementation can be given at early stages of infections.

Minerals

The electrolytes (sodium, potassium and chloride) are lost excessively through perspiration during fever. In typhoid patients, iron supplementation is needed for patients to prevent anemia. Calcium and phosphorus supplementation is given for tuberculosis patients who are taking isoniazid drugs.

Fibre

Low fibre diet is given for the patients. The patient should be given 20-25g/day. In typhoid, intestinal lesions and diarrhoea may occur.

Fluid

Hydration is important during fevers. It is advisable to consume 2.5 to 3.5 liters of fluid per day. This includes juices, soups, thin dal, broths and water.

Other considerations

- Semi solid diet and Soft diet are desirable to improve appetite. This will also help the patient to consume nutritionally adequate diet.
- Small frequent meals can be given. Six to eight feedings will be sufficient.
- Less spicy foods are preferred.

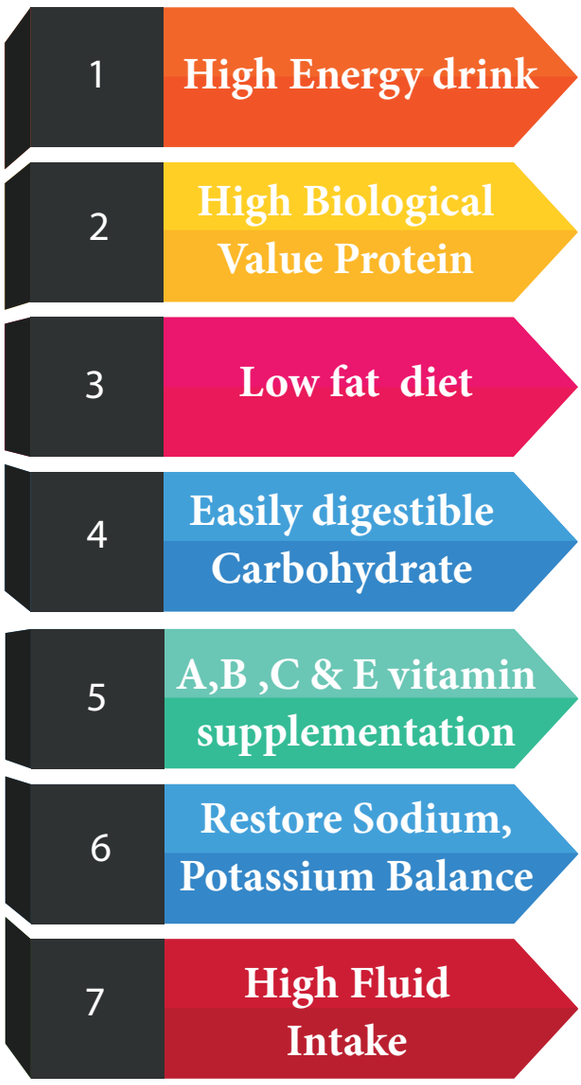


Fig 6.3 : Diet Therapy for Fevers

UNIT - 6 DIET IN FEVERS.indd 106 25-02-2019 15:35:36